

A 4-PART POLICY SERIES ON ONTARIO
HOSPITAL MENTAL HEALTH AUTHORITY

THE SPACE BETWEEN.

The Continuous Authority Gap in Ontario's Hospital Mental Health Crisis Response, the Fiscal and Operational Case for Reform, and a Proposed Ottawa Pilot Under the Community Safety and Policing Act.

THESIS

A person in mental health crisis who arrives at a hospital should not face delays in care because no one on site has the authority to help them safely access clinical assessment.

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SERIES	PAPERS	YEAR
The Space Between	4 Papers	2026

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Ontario



The Rt. Hon. Mark Carney – Prime Minister of Canada
& MP for Nepean

The Hon. Doug Ford – Premier of Ontario

// EXECUTIVE SUMMARY

The Problem, the Solution, and Why It Matters.

Ontario's hospital mental health crisis response is structurally dependent on a single external variable: timely police invocation of Section 17 of the Mental Health Act. When that variable fails — through delay, capacity constraint, or discretionary non-invocation — no professional on site holds the legal authority to bridge a patient in crisis to timely clinical assessment. This report documents that gap, quantifies its cost, and proposes a regulated, safeguarded, Ottawa-based pilot to close it.

// THE PROBLEM

A CONTINUOUS AUTHORITY GAP ACROSS THREE STAGES.

Ontario hospitals face authority vacuums before triage, between triage and physician assessment, and during clinical escalation. In 45.4% of Ontario psychiatric ED visits, patients arrive from outside the system entirely (JAMA Network Open, 2021, n=659,084). Involuntary admissions rose to 55.7% in 2021 (European Psychiatry, 2024, n=9,848). Mental health ED presentations rose 47% in recent years (CMHA, 2024).

// THE SOLUTION

A HEALTHCARE SPECIAL CONSTABLE DESIGNATION.

Under the existing *Community Safety and Policing Act*, the Ottawa Police Services Board can designate trained hospital protection services officers as Special Constables with narrow, geographically bounded authority to initiate a Section 17 apprehension where legally justified. The mechanism is operational in Ottawa today through OC Transpo. No new legislation is required.

COST, SAFETY, AND SYSTEM STRAIN.

Police time spent in Ottawa hospitals on mental health handovers is conservatively \$800K per year at baseline. ED disruption, repeat presentations, and workforce strain driven by violence exposure compound the cost. Mental health and substance use ED visits in Ontario are rising. The authority gap becomes progressively more complex, more costly, and more clinically risky over time.

OTTAWA TWO-SITE PILOT.

An Ottawa two-site pilot — one community-acute hospital and one academic-teaching hospital — 18 months, 40 designated officers maximum, strict scope limitations, mandatory body-worn cameras, clinical primacy enforcement framework, independent academic evaluation, and binding off-ramp conditions. Continued funding tied to measurable outcomes, limiting fiscal exposure.

// ONE-LINE TAKEAWAY

This proposal focuses on using existing system resources more efficiently rather than introducing new costs. It asks Ontario to close a gap that is already producing preventable harm, using a legislative mechanism that already exists.

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PART I

The Problem. ■

// 1.0 — THE AUTHOR'S POSITION

Written From the Floor.

This report is written by a Protection Services Officer. A decade in security. The past four years in hospital protection services in Ontario. It is not written from a think tank, a faculty office, or a policy institute.

It is written because the incident that prompted it was not the first, will not be the last, and is happening in Ontario hospitals tonight.

// THE INCIDENT

A woman arrived at the hospital in acute psychiatric crisis. A knife had been disclosed in her vehicle. Police response took approximately ninety minutes. When officers arrived, a decision was made not to initiate a Section 17 apprehension. The interaction escalated. She headbutted an officer. She was arrested. She left the hospital not in clinical care but in a police vehicle. She came to a hospital. In Ontario, that was not enough.

This is the first of four papers in a consolidated policy series. Its purpose is to name — precisely, and without overreach — a gap in Ontario's legislative framework that produces this outcome more often than the system acknowledges.

// TAKEAWAY

The gap described in this report is not theoretical. It is a daily operational reality in every major Ontario hospital, written here by a person who has lived inside it.

// 1.1 — THE THREE-STAGE FAILURE MODEL

The Gap Is Continuous.

The authority gap is not a single moment. It is continuous across three distinct stages of the hospital mental health encounter. Each has its own authority vacuum. Each produces its own pattern of preventable harm.

Stage 1 — Pre-triage

A person arrives on hospital property in acute mental health crisis. They have not been triaged. They have not been registered. They may be in the parking lot, the main entrance, the ambulance bay, the waiting area. They are clearly in distress. They have not entered the clinical pathway.

- The **physician** cannot assess them — no clinician-patient relationship exists yet.
- The **protection services officer** cannot act on mental health grounds — authority under the *Criminal Code* and *Trespass to Property Act* does not extend to psychiatric presentation absent criminal conduct.
- The **triage nurse** cannot physically compel cooperation.
- Only **police** hold Section 17 authority — and they must be dispatched, arrive, and elect to invoke it.

This is the pre-triage window. It is the space between arrival and care.

Stage 2 — Post-triage, Pre-assessment

The person has successfully engaged with triage. They are in the clinical system. They are waiting to be seen by a physician.

Ontario emergency departments operate under the Canadian Triage and Acuity Scale, which classifies most psychiatric presentations as CTAS Level 3 — a classification confirmed in a 2012 *Canadian*

Medical Association Journal study of Ontario emergency departments. CTAS Level 3 means urgent but not immediately life-threatening. In practice: waits of hours, not minutes.

During those hours, the patient may deteriorate. De-escalation may fail. A verbal incident may become physical. The clinical authority to initiate a Form 1 requires physician assessment, which has not yet occurred. The protection services officer can respond to the incident if it becomes criminal, but has no authority to preemptively escalate care.

Stage 3 — Escalation During Care

The person has been assessed. A Form 1 may have been issued. Clinical care is underway. The scenario that Ontario Hospital Association Code White protocols are designed to manage — behavioural emergency involving immediate risk of harm to self or others — is now in progress.

Code White responds. Protection Services arrives. Clinical team manages the patient. The authority framework operates, at this stage, at its strongest. But the authority that Code White can exercise is still reactive and fundamentally criminal — not preventive and clinical.

// STRUCTURAL OBSERVATION

EACH STAGE HAS ITS OWN AUTHORITY VACUUM.

The physician's authority begins at assessment. The protection services officer's authority begins at criminal conduct. The police officer's authority is external, discretionary, and time-variable. No single actor holds continuous authority across all three stages. The patient experiences the gap as a single continuous delay. The system experiences it as three separate problems.

// TAKEAWAY

The gap is continuous. Any solution addressing only one stage leaves the other two unmanaged. This is why the proposal must be structural, not operational.

Who Can Act, and When.

The following map describes precisely what each actor in a pre-triage Ontario hospital mental health scenario is legally empowered to do. It is drawn directly from the governing statutes.

CONDITIONAL — POST-TRIAGE ONLY

Physician / Clinical Staff

Form 1 authority under MHA s.15 — requires completed assessment. Form 3 / 4 for continued involuntary admission. *Health Care Consent Act* governs treatment, not pre-triage access. Authority is clinically triggered and requires system entry first.

Gap: Clinical powers begin at assessment. Assessment requires triage. Triage requires system entry. System entry cannot be compelled without police or criminal grounds.

CRIMINAL LAW ONLY — NO MHA TOOLS

Protection Services Officer

s.494 *Criminal Code* — citizen's arrest for crime committed or imminent. *Trespass to Property Act* — occupier's authority to remove. Zero authority under the *Ontario Mental Health Act*. De-escalation, verbal intervention, observation only on mental health grounds.

Gap: Cannot act on mental health crisis absent criminal conduct. Trespass removal in an acute psychiatric presentation is clinically contra-indicated and patient-harmful.

FULL MHA AUTHORITY — EXTERNAL & DISCRETIONARY

Police Officer

s.17 MHA — full apprehension authority for persons in mental health crisis. Full *Criminal Code* authority in parallel. Invocation is discretionary — officer must personally form grounds. No guaranteed response SLA for mental health calls to hospitals.

Gap: Authority exists — but is external, discretionary, and not reliably available. Every hospital emergency plan depends on this single external variable performing correctly.

// POLICE ROLE CLARITY

POLICE RETAIN FULL SECTION 17 AUTHORITY. NOTHING IN THIS PROPOSAL ALTERS IT.

This proposal is not a substitute for police response. It is a recognition that operational demands, competing priorities, and the volume of mental health calls province-wide can limit the ability of any police service to provide consistent, timely, on-site response within hospital environments.

This approach allows police to focus on higher-priority community calls requiring full police authority, while ensuring that patients already on hospital property are not left without access to care during periods when police response is delayed or operationally inappropriate for the clinical setting.

The Ottawa Police Service's 2025 Crisis Intervention Team program — supported by a \$4M Solicitor General grant — is itself a formal institutional acknowledgment that existing police capacity requires supplementation. The Healthcare Special Constable designation complements that direction. It does not replace it.

// TAKEAWAY

The legislative map contains an irreducible structural gap. Only police hold the continuous authority required — and only police operate under conditions that cannot guarantee timely, consistent invocation.

// 1.3 — WHO IS BEING HARMED

The Population At the Gap.

The people falling through the authority gap are the most vulnerable patients in Ontario's mental health system.

45.4%

Of Ontario psychiatric ED visits are first-contact — no prior outpatient mental health contact. Nearly half arrive completely outside the system.

Kurdyak et al. JAMA Network Open, 2021. n=659,084.

55.7%

Of Ontario psychiatric inpatients in 2021 were admitted involuntarily, up 6.8 percentage points from prior year. Rate continues to rise.

Fung & Kim. European Psychiatry, 2024. n=9,848.

+47%

Increase in Ontario ED visits for mental health, addictions, and substance use. Youth ages 14-17 hospitalisation rose 136%.

CMHA State of Mental Health in Canada Report, 2024.

+15%

Growth in OPS mental health calls since 2019. Occurrences with a mental health component rose 30% in the same window.

The patients most affected by the gap are not statistical abstractions. Peer-reviewed Ontario data documents that **Indigenous persons, low-income persons, rural residents, and persons in unstable housing are disproportionately represented in involuntary psychiatric admissions in this province** (Fung & Kim, 2024). The authority gap does not harm the population evenly. It harms the population that is already least well served.

The Inevitability Frame

The data does not suggest this challenge will stabilise on its own. Ontario's Auditor General has documented that the Ministry of Health lacks a health human resources strategy adequate to meet growing mental health demand. The 2025 OPS CIT launch is itself a formal institutional acknowledgment that existing capacity is insufficient.

// INEVITABILITY — THREE AXES

On the current trajectory, the authority gap becomes progressively more complex, more costly, and more clinically risky over time. **The fiscal exposure grows** as case volume grows. **The operational complexity grows** as the system adds more downstream mitigations — crisis intervention teams, mobile crisis units, paramedic diversion programs — without closing the on-site gap itself. **The clinical risk grows** as the population arriving at hospitals in first-contact psychiatric presentations continues to rise. Inaction is not a neutral choice. It is a choice that carries measurable and increasing cost across all three dimensions.

// TAKEAWAY

The gap harms the patients least able to absorb further system failure — and the cost of leaving it unaddressed grows every year.

A Clinical Bridge.

The solution is narrow and specific. A **Healthcare Special Constable designation** under the existing *Community Safety and Policing Act*, administered by the Ottawa Police Services Board with Solicitor General approval. The same mechanism that governs OC Transpo Special Constables today.

The designation grants trained protection services officers the narrow authority to initiate a Section 17 apprehension where legally justified — only within defined hospital property, only in scenarios meeting the Section 17 threshold, and only as a bridge to timely clinical assessment.

- This is **not an expansion of enforcement authority**. It is a patient-centred clinical bridging mechanism.
- It does **not replace police**. Police retain full Section 17 authority; the designation complements existing capacity.
- It does **not override clinical judgment**. Where clinicians are available, the clinical pathway has priority. Clinical staff retain real-time override authority at all times.
- It operates under **strict scope limitations, mandatory documentation, body-worn cameras, and independent oversight**.
- It is **tested, time-limited, and terminable** through a 12-18 month Ottawa pilot with binding evaluation criteria.

The remainder of this report documents the structural conditions that make this solution necessary (Part I, continued), the fiscal case (Part II), and the complete implementation framework with full Charter and clinical safeguards (Part III).

// THE ETHICAL ANCHOR

A person in mental health crisis who arrives at a hospital should not face delays in care because no one on site has the authority to help them safely access clinical assessment.

How the Designation Operates.

The authority gap described so far occurs most often in one of two operational situations: a person arrives at hospital property independently in pre-triage crisis, or police arrive with a person already apprehended under Section 17 and are required to remain on site, sometimes for hours, until a physician completes the assessment. The first situation produces the harm to the patient. The second situation produces the resource drain on policing and the criminalisation experience for the patient. The Healthcare Special Constable designation closes both.

How the Offload Works in Practice

When an Ottawa Police Service officer arrives at a pilot hospital with a person already apprehended under Section 17, the designated Healthcare Special Constable on duty assumes continued custodial authority for that person at the point of hospital entry. The OPS officer transfers the Section 17 apprehension responsibility through a defined handover protocol — identification of the patient, documented grounds for apprehension, any relevant risk information, and transfer of continuous custody to the Healthcare Special Constable who now holds the on-site Section 17 authority within the hospital jurisdiction.

From that moment, the OPS officer is free to return to service. The Healthcare Special Constable remains with the patient until clinical assessment, with the on-duty hospital protection services team available as assist resource if the situation requires additional presence. The patient continues in an MHA pathway, not a criminal one. The officer returns to the road.

// THE OPERATIONAL MODEL IN ONE PARAGRAPH

OPS brings in the Section 17. The Healthcare Special Constable assumes custody at hospital entry. The OPS officer returns to service. Hospital Protection Services backs up the Special Constable if the situation requires. The patient is met by someone with the authority and the clinical integration training to bridge them to assessment. Police time is released. Patient experience improves. The apprehension remains within the MHA framework from start to finish.

What This Resolves

- **Police resource reallocation.** The primary fiscal benefit of the designation. OPS officers no longer wait in hospital EDs for extended periods — they hand over, document, and return to patrol. The 2025 OPS CIT program has already documented this effect with non-hospital special constables (250 officer-hours freed per platoon, 2.5 additional calls per day per officer relieved). The hospital designation produces the same effect on the hospital mental health call subset specifically.
- **Continuity of MHA pathway.** The person remains in a mental health apprehension pathway under Section 17 throughout — they are never held under criminal authority as a consequence of prolonged police presence. Clinical bridging is preserved as the primary function.
- **Reduction of criminalisation optics.** HSJCC 2019 and patient advocacy literature consistently identify uniformed police presence in hospital waiting areas as harmful to patient experience and recovery engagement. The Healthcare Special Constable operates in the clinical environment with clinical integration training and a designation that is visibly hospital-based rather than police-based.
- **Pre-triage capacity.** Because the Healthcare Special Constable is already on site 24/7, the designation also closes the pre-triage gap for walk-in presentations — where OPS is not yet involved and would otherwise have to be dispatched. This is the dual function: police offload where police are present, on-site bridging where police would otherwise be called.

The Tiered Response Model

The operational framework envisioned is a tiered response in which the primary authority resides with whichever actor is most appropriate to the specific moment. Police hold full Section 17 authority and remain the primary responders for community apprehension and transport. The Healthcare Special Constable holds jurisdictionally limited Section 17 authority on hospital property and operates as the clinical bridging actor. Hospital Protection Services continues to operate under existing Criminal Code, Trespass to Property, and Code White authorities and provides assist resource, particularly in dynamic or multi-patient scenarios.

At no point are police replaced. At no point does the Healthcare Special Constable operate outside their defined jurisdiction. At no point does Hospital Protection Services act outside the existing legal framework that governs their work today. The model layers three roles that already exist into a tiered response that closes the gap each role cannot close alone.

// TAKEAWAY

The designation operationalises a simple handover: police arrive with a Section 17, a Healthcare Special Constable assumes on-site custody, police return to service, Hospital Protection Services

assists as needed, and the patient bridges to clinical assessment without leaving the mental health care pathway. This is the operational mechanism. Part II documents what that mechanism is worth.

// 2.0 – THE THREE STAGES IN EMERGENCY MANAGEMENT TERMS

Ontario's Planning Gap.

Ontario hospitals are legally required to maintain emergency management programs under the **Emergency Management and Civil Protection Act, RSO 1990, c E.9 (EMCPA)**. These programs must address foreseeable threats to patient safety, staff welfare, and operational continuity. They are tested annually. They are accreditation-reviewed.

In every Ontario hospital emergency plan, the same assumption is embedded: when a person in mental health crisis presents on hospital property in a way that threatens safety, police will be called, and police will act.

That assumption describes the status quo. It does not survive stress analysis.

The Code White Problem

The Ontario Hospital Association's Code White protocol is the standardised hospital response to a behavioural emergency involving immediate risk of harm. Every Code White protocol in Ontario addresses **escalation within the clinical environment** — Stage 3 in the three-stage model. University Health Network's published Code White policy defines the trigger as "any individual within hospital boundaries behaving in a potentially dangerous manner towards themselves or others."

No Code White protocol governs Stage 1 (pre-triage arrival) or Stage 2 (post-triage, pre-assessment waiting room deterioration). The reason is not operational oversight. The reason is legislative foundation. You cannot write a protocol for authority you do not have.

// STRUCTURAL OBSERVATION

Ontario hospital emergency plans have a well-developed response for mental health crises that occur inside the clinical environment. They have no response for mental health crises that arrive at the

One External Variable.

Emergency management doctrine identifies **single points of failure (SPOFs)** as the highest-priority risk factor in any operational system. A SPOF is a component whose failure alone causes the entire system to fail. Sound emergency planning requires that critical dependencies either have redundant backup mechanisms or be designed out of the system entirely.

Ontario hospital emergency management plans have a SPOF in their pre-triage mental health response pathway. It is police response.

The dependency failure matrix below is drawn directly from Ontario legislative authority analysis.

CRITICAL DEPENDENCY	REQUIRED FOR	BACKUP MECHANISM	RISK
Police Section 17 invocation	Legal authority to act on pre-triage mental health crisis	None — no on-site equivalent exists in Ontario law	CRITICAL
Timely police response	Operationalising Section 17 within a clinically relevant window	None — no SLA governs mental health response time to hospitals	CRITICAL
Police decision to invoke	Converting reasonable grounds into actual apprehension	None — officer discretion cannot be compelled or appealed in real time	CRITICAL
Patient voluntary cooperation	Triggering clinical authority for physician assessment	Partial — de-escalation; unreliable in acute presentations	HIGH
Physician Form 1 authority	Formal legal authority for involuntary psychiatric assessment	Robust — multiple physicians, well-documented process	LOW

Every critical dependency in the pre-triage mental health pathway is either unmitigated or dependent on conditions that cannot be controlled. The one element robustly supported — physician Form 1 authority — only becomes relevant after the pre-triage window has closed.

The Gap Is Confirmed Nationally.

British Columbia — Victoria Police Chief on Record

In September 2024, Victoria Police Chief Del Manak held a press conference to disclose that his officers had recently waited **seven hours** at Royal Jubilee Hospital to transfer a patient apprehended under the *BC Mental Health Act* to hospital care. Average police wait times at the facility grew from 1 hour 47 minutes the prior year to 2 hours 20 minutes in summer 2024 — reaching eight hours in extreme cases.

// CHIEF DEL MANAK, VICPD, SEPTEMBER 2024

"If the government was able to restore special constable status to hospital security... that will allow the transference of individuals that are apprehended immediately, or quite immediately, for our officers to return to the streets."

Victoria Mayor Marianne Alto confirmed the problem had been occurring for years. Island Health's vice-president of clinical services stated the designation was "currently being assessed at a provincial level." BC Health Minister Adrian Dix stated the Ministry had been working with Victoria police on the issue.

Union of BC Municipalities — Formal Resolution

The UBCM passed a formal resolution calling on the provincial government to amend the *BC Mental Health Act* to give trained hospital security staff special constable status and peace officer status. The

resolution explicitly identified the solution as empowering hospital security staff with the legal authority to assume responsibility for persons in mental health crisis — removing the police dependency for that specific function.

Alberta — The Model in Operation

Alberta Health Services operates a Community Peace Officer program within its hospital system under the *Alberta Peace Officer Act, RSA 2006, c P-3.5*. Operational for over a decade, across AHS facilities serving nearly five million Albertans. Six-week paid induction administered jointly with Alberta Justice and Solicitor General. Officers carry the authority to act on mental health presentations before clinical assessment is initiated.

Ontario — Ottawa, Documented Locally

A 2009 collaboration between Ottawa Police Services and The Ottawa Hospital — documented in the Ontario Human Services and Justice Coordinating Committee's 2013 report and in *Healthy Debate* — was established because OPS officers were spending unacceptable time in hospital EDs.

// OTTAWA — THE DOCUMENTED "DANGEROUS MYTH"

The 2014 *Healthy Debate* analysis, drawing on Ottawa-specific data, documented that police officers routinely wait unnecessarily in EDs due to **"a common myth among hospital staff, security contractors, and even police"** that a physician's Form 1 determination is required before security can assume patient monitoring. The authors described this as **"a dangerous myth because tragedies have occurred in the interval between patient arrival and physician assessment."**

The myth endures because no clear legislative authority exists for anyone other than police to manage that interval.

// TAKEAWAY

The pre-triage authority gap is not a local complaint. It is a documented systemic failure across multiple Canadian jurisdictions. The solution — special constable or peace officer status for trained hospital security — is operational in Alberta and being actively requested by police leadership in BC.

A Structural Lesson.

In 2019, Paul Spencer died at Royal Jubilee Hospital in Victoria, British Columbia, following an altercation with hospital security guards during his involuntary psychiatric care. The BC Coroner's Service opened an inquest into the circumstances of his death in September 2024.

The Spencer case is the single most serious patient-safety incident referenced in the Canadian hospital protection services discussion. It must be engaged directly and honestly.

What the Case Reveals

The conditions identified in the review of Spencer's death included:

- No provincial legislative framework governing hospital security authority in psychiatric crisis
- No regulated use of force standard specific to clinical settings
- No mandatory body-worn camera requirement during such interactions
- No independent oversight body with specific jurisdiction over hospital security clinical conduct
- No mandatory adverse event review protocol

Hospital security personnel in British Columbia had lost special constable status more than twenty years earlier. They operated in a legislative and regulatory vacuum. The tragic outcome was not the result of security personnel having too much authority. It was the result of security personnel operating in a structural vacuum — called upon to manage clinical crises for which no framework governed their training, their conduct, their oversight, or their accountability.

// THE CORE LESSON

The absence of a regulated framework — rather than the presence of authority — is what creates the greatest risk in clinical security interactions. Unregulated discretion, operating without defined standards, without independent oversight, and without mandatory review, is the condition most strongly associated with adverse outcomes.

The Ontario Condition

Ontario's current hospital protection services operate under several of the same structural conditions. Officers manage pre-triage, post-triage pre-assessment, and escalation scenarios involving patients in psychiatric crisis as a regular part of their work, and they do so without the provincial legislative framework, regulated use of force standards, mandatory recording requirements, or adverse event review infrastructure that would support them.

This is not a reflection on the hospitals that employ them or on the officers themselves. It is a reflection of a legislative framework that has not yet been built to govern the environment they actually work in.

How the Proposed Designation Reduces Spencer-Level Risk

Every element of the Healthcare Special Constable framework proposed in this series is a direct response to a condition that was absent in the Spencer case:

- **Legislative framework.** CSPA Part XIV Special Constable designation places officers within a defined legal authority structure with explicit obligations, limits, and oversight.
- **Defined use of force standards.** Training to Ontario Police College standards, clinically adapted, with continuum requirements identical to or exceeding the policing baseline.
- **Mandatory body-worn cameras.** Continuous recording during any apprehension or use of force, with retention and review protocols.
- **Independent oversight with real jurisdiction.** CSPA Part XIV oversight plus Law Enforcement Complaints Agency jurisdiction plus OPSB review plus independent academic pilot evaluator.
- **Mandatory adverse event review.** Automatic review triggers defined in the Pilot Containment framework.
- **Patient rights safeguards.** Full MHA patient rights framework applies, including right to rights advisor and right to Consent and Capacity Board review.
- **Disaggregated equity auditing.** Quarterly race-based, Indigenous-status, and disaggregated reporting to prevent discriminatory drift.

The Spencer case is not an argument against the Healthcare Special Constable designation. It is the clearest available argument for the specific safeguards this series proposes. Those safeguards are designed precisely to prevent the reproduction of the conditions that led to his death.

// 2.4 — ALTERNATIVES CONSIDERED

What Else Was Considered.

Responsible policy analysis requires a clear accounting of alternatives. Four were evaluated.

Alternative 1 — Increased police presence in hospitals

Stationing police at major EDs would provide direct Section 17 authority. The approach works mechanically. The limitation: provincial policing capacity is already insufficient for existing demand (Auditor General 2023; OPS CIT 2025). Policing cost is significantly higher — OPS first-class constable \$111,000+ base vs. OC Transpo special constable \$80,893–\$95,168. Stationing police in clinical environments reproduces the criminalisation framing that HSJCC 2019 and OHRC have identified as harmful.

Does not resolve the gap — reproduces the current dependency at higher fiscal and patient-care cost.

Alternative 2 — Expanded clinical authority for nurses or physicians

Granting specified clinicians authority to initiate involuntary assessment prior to triage would shift the pathway into the clinical framework. Addresses Stage 2. Does not address Stage 1 at all — the *HCCA* and professional regulatory frameworks require a clinician-patient relationship to exist first. Would require amending the *HCCA*, *Regulated Health Professions Act*, and multiple professional standards — an order of magnitude more complex than adapting existing CSPA infrastructure.

Addresses one stage at the cost of exponentially larger legislative reform.

Alternative 3 — Mobile crisis teams alone

Expanding mobile crisis teams — including OPS CIT — improves response quality when crises are identified in the community. Valuable and should continue. Mobile teams operate in the community; they do not address crises that present at hospital property and do not remain on-site 24/7.

Complementary, not substitutive. Does not close the on-site hospital gap.

Alternative 4 — Status quo

Maintaining existing arrangements continues a documented failure (Auditor General 2023, 2025; Kurdyak 2021; Fung & Kim 2024; CMHA 2024). The 2025 OPS CIT launch is itself institutional acknowledgment that the status quo is insufficient.

The status quo is not a neutral option. It is a choice to continue a documented failure.

// WHY THE PROPOSED MODEL IS THE LEAST DISRUPTIVE TARGETED SOLUTION

A Healthcare Special Constable designation under the existing CSPA addresses the continuous authority gap across all three stages using a legislative mechanism operational in Ottawa today. It requires no new legislation — only application of existing CSPA Part XIV authority by the Ottawa Police Services Board. It does not expand police deployment. It does not require amendment of the *HCCA* or any regulated health profession statute. It does not replace mobile crisis teams, CIT programs, or upstream mental health investments. It adds the missing on-site, 24/7, care-access bridge that none of the alternatives provide.

// 2.5 — THE EMCPA MODERNISATION WINDOW

Ontario Is Already Rewriting the Rules.

Ontario is presently in the most significant modernisation of its emergency management legislative framework since the EMCPA replaced the *Emergency Management Act*. **Bill 25, the Emergency**

Management Modernization Act, 2025, amends the EMCPA to strengthen provincial leadership, expand community capacity, and require critical infrastructure operators to develop emergency management programs. The **Ministry of Emergency Preparedness and Response** — created March 19, 2025 — is the new coordinating body.

The modernisation consultation involved more than 480 partners, 14 in-person and 31 virtual engagements, and 90 written submissions. The pre-triage authority gap was not raised. It does not appear in Bill 25 or proposed regulations.

This series, when submitted, constitutes that formal gap disclosure.

// TAKEAWAY

The EMCPA modernisation window is open. A formal submission identifying the pre-triage authority gap as an unmitigated critical dependency in hospital emergency management is the most direct available pathway to provincial acknowledgment.

PART II

The Cost of Inaction.

A fiscal and operational analysis of what the authority gap currently costs Ontario's healthcare and public safety systems, what the proposed pilot would cost, and the break-even threshold the pilot must meet.

The Numbers Behind the Gap.

This analysis rests on a single governing premise.

// THE POLITICAL-LEVEL FRAME

This proposal focuses on using existing system resources more efficiently rather than introducing new costs. This is not about spending more. It is about spending better.

The cost of the gap is measured across four categories: direct operational costs (police time, ED disruption), human capital costs (workforce strain, turnover driven by violence exposure), patient pathway costs (repeat presentations), and liability costs. Implementation is measured against these four.

// TREASURY CONTROL SIGNAL

The pilot structure ensures that continued funding is contingent on measurable outcomes, limiting fiscal exposure. Independent evaluation at the conclusion of the pilot produces binding recommendations tied to pre-defined performance criteria. Provincial investment beyond the pilot is earned through demonstrated performance, not assumed through implementation momentum.

What Hospital Mental Health Calls Cost OPS.

Ottawa Police Cost Per Hour

- OPS first-class constable base salary (2024 collective agreement, CBC News Dec 2024): **\$111,000+**
- 19.35% increase scheduled through 2029
- Annual FTE hours: 2,080
- Base hourly: $\$111,000 \div 2,080 = \textbf{\$53.37/hour}$
- Loaded cost at 1.5x multiplier (standard public sector estimate for benefits, overhead, equipment): approximately **\$80/hour**

Labeled assumption: 1.5x loaded cost multiplier is a standard public sector estimate. Sensitivity range 1.3x–1.7x.

Mental Health Apprehension Volume

- Toronto Police: 8,500 MHA apprehensions in 2011 (CMAJ, HSJCC 2013)
- OPS mental health calls: +15% since 2019 (OPS CIT Report 2025)
- OPS occurrences with a mental health component: +30% since 2019
- Modelled Ottawa mental health apprehension volume (conservative): **2,000–3,000 per year**

Police Time Per Call

Benchmarked to BC VicPD documented figure (2.5 hours average per handover; Chief Manak press conference Sept 2024).

Labeled assumption: Average Ottawa police time per hospital mental health call approximates the BC documented figure. Actual Ottawa figure not publicly published.

Annual Cost of Current Police Dependency — Ottawa

Scenario	Volume	Hours	Officers	Annual Cost
Conservative Low	2,000 apprehensions	2.5 hours each	2 officers	\$800,000

Moderate	2,500 apprehensions	3.0 hours each	2 officers	\$1,200,000
High	3,000 apprehensions	3.5 hours each	2 officers	\$1,680,000

// INSTITUTIONAL VALIDATION

The \$4M Solicitor General grant to OPS for CIT special constables in 2025 is itself a documented government acknowledgment of the scale of this cost. Ontario has already spent \$4M responding to the inadequacy of existing police mental health response capacity.

// 3.2 – THE POLICE OFFLOAD BENEFIT

The Fiscal Mechanism, in Operational Terms.

Section 3.1 calculated the annual cost of existing police time consumed by hospital mental health handovers. Section 3.2 describes the mechanism by which a Healthcare Special Constable designation converts that cost into measurable reallocation.

The Mechanism

When an OPS officer arrives at a pilot hospital with a Section 17 apprehension, the designated Healthcare Special Constable on duty assumes custodial authority at the point of hospital entry. The OPS officer transfers the apprehension through the defined handover protocol and is free to return to service. The patient continues in the MHA pathway with on-site Section 17 authority maintained by the Special Constable, supported by Hospital Protection Services as assist resource where required.

The fiscal effect is not a modelled savings. It is a direct, auditable reallocation. An OPS officer who today spends 2.5–3.5 hours completing a hospital mental health handover spends instead a defined handover window (proposed target: 15–20 minutes) and returns to patrol. The saved officer-hours flow back to community response immediately.

The OPS CIT Precedent

The Ottawa Police Service has already documented this effect at operational scale in a different context.

The 2025 OPS CIT special constable pilot produced the following measured outcomes:

- **250 officer-hours freed per platoon** in the pilot period
- **2.5 additional calls per day** attended by each relieved officer
- Platoons supported by special constables devoted more time to proactive policing
- Funded by a **\$4M Solicitor General grant** in formal recognition of the resource reallocation logic

The OPS CIT program is not the same intervention as this proposal. It relieves police of one of two officers on mental health calls generally; this proposal relieves police of both officers specifically on hospital handovers. But the operational logic is identical: special constables, appointed under the same CSPA framework, free police capacity that is currently consumed by calls that do not require full police authority.

The Offload Model as the Primary Fiscal Anchor

// WHY THIS IS THE PRIMARY ANCHOR

The break-even threshold in Section 3.7 rests on police time reallocation because it is the most defensible offset in the model. Police compensation is publicly disclosed (Ottawa Sunshine List). Loaded hourly rates are standard public sector accounting. The OPS CIT pilot has already demonstrated that special constable designations produce measurable police time reallocation. Every other offset category in this analysis is directionally significant but requires more modelling. The police offload benefit is the one the Treasury Board can verify in a single briefing note.

The Tiered Response Fiscal Picture

The designation does not add a new category of public spending. It redistributes workload across three roles that already exist and are already funded:

- **Police** remain the primary responders for community apprehensions and remain fully funded at current levels. Their capacity is released, not reduced.

- **Healthcare Special Constables** are drawn from existing hospital protection services personnel through designation, not through net new hiring. Existing officers gain the legislative authority to perform the bridging function they are already improvising around today.
- **Hospital Protection Services** continues operating under its current authority framework and provides assist resource to the designated officer in dynamic scenarios.

The proposal uses existing personnel, existing authority mechanisms, and existing oversight infrastructure. The pilot cost is the cost of training, equipment, body-worn cameras, and independent evaluation — not the cost of new public-safety capacity.

// TAKEAWAY

The police offload benefit is the primary fiscal argument. It is the offset the OPS CIT pilot has already proven operationally. It is the number the Treasury Board can verify. It is the mechanism the designation exists to produce.

// 3.3 — ED DISRUPTION AND WORKFORCE STRAIN

The Costs Inside the Hospital.

ED Disruption — What Is Directly Supportable

ED disruption is directionally significant but not directly costed in Ontario's public reporting. The supportable elements of the argument are as follows.

- **CIHI NACRS** reports more than **16.1 million unscheduled ED visits** in Canada in 2024–25, up from almost 15.5 million in 2023–24.
- **Ontario's Auditor General (2023)** documented severe ED staffing strain at reviewed hospitals — including one hospital spending approximately **\$8 million on agency nurses for its emergency department in 2022/23**, up from \$2.4 million the prior year and approximately \$1 million the year before that.

- The same audit documented significant deterioration of ED nursing vacancy rates at reviewed sites between 2019/20 and 2022/23 — including one ED where full-time RN vacancy rose from approximately **6% to 26%** and part-time RN vacancy from approximately **23% to 51%**.
- **CADTH** documents that ED overcrowding overworks staff, contributes to burnout, puts patient health at risk, and adds strain to the broader health system.
- Published Canadian emergency medicine research documents that **mental health ED presentations experience longer waits and longer lengths of stay than non-mental-health presentations**. A peer-reviewed study published in *CMAJ* found patients with mental illness waited significantly longer for physician assessment after adjustment for other variables. A separate US academic medical centre study (Nicks and Manthey, 2012) documented that psychiatric ED length of stay was **3.2 times longer** than non-psychiatric admissions (1,089 minutes vs. 340 minutes).

ED Disruption — What Is a Modelled Assumption

Beyond the supportable elements above, the following are structured assumptions. They are included because an incomplete fiscal case is less useful than a transparently labeled one, but they are not represented as published findings.

- *Labeled assumption:* Average disruption cost per prolonged pre-triage mental health ED presentation modelled in the range of **\$500–\$1,200 per incident**, anchored to the Nicks and Manthey 3.2x length-of-stay multiplier applied to CIHI's documented \$397 average Canadian mental health ED visit cost (2018–19), and cross-checked against the Alameda Model Psychiatric Emergency Service billing rate of \$97–\$140 per patient-hour for crisis stabilization in comparable US settings. Actual Ontario-specific per-incident figure not publicly published.
- *Labeled assumption:* A portion of prolonged pre-triage presentations would be bridged earlier under the designation, with associated reduction in disruption length. Specific percentages require the pilot itself to validate.

Applied to the modelled Ottawa volume of 2,000–3,000 prolonged pre-triage incidents per year, the defensible modelled range is **\$1M–\$3.6M annually**, with more weight toward the lower end under conservative assumptions. This figure functions as a secondary offset in the break-even case; the primary anchor remains police time reallocation (Section 3.2).

ED disruption is a material but directionally modelled cost category. The directly supportable elements — rising ED visit volume nationally, documented staffing strain in Ontario EDs, longer LOS for mental health patients — establish the pattern. The per-incident dollar figure remains a structured assumption until the pilot produces Ontario-specific data.

Workforce Strain and Violence Exposure

- Ontario Nurses Association 2021: approximately 70% of nurses report witnessing or experiencing physical violence at work
- Canadian Federation of Nurses Unions: approximately 1 in 5 nurses experience physical violence in the workplace
- CCOHS: healthcare workers 5x more likely to experience workplace violence than other sectors
- OCHU-CUPE 2024: 331% increase in hospital job vacancies since 2015
- WorkSafeBC 2024: 170 accepted injury claims for BC hospital security guards in one year — "injured nearly every other day" (CBC News Aug 2025)

Labeled assumption: Ontario RN replacement cost approximately \$70,000–\$90,000 CAD. Based on directional adjustment of NSI 2024 US benchmark (\$61,110 USD). Actual Ontario-specific replacement cost not publicly consolidated.

Labeled assumption: Violence-driven turnover component: conservatively 10–15% of overall healthcare turnover per CFNU and CCOHS framing.

// DATA GAP AS A FINDING

Canadian healthcare worker assault and turnover data is not systematically collected at the provincial or national level. This is both a limitation of the analysis and a recommendation of Paper 4: the establishment of a provincial hospital protection services data framework as part of the pilot.

What Failed First Contact Costs.

Kurdyak et al. (2021) documented that **45.4% of Ontario psychiatric ED visits are first-contact** — no prior outpatient mental health contact in the preceding two years. The pre-triage gap means these patients either do not enter the system or enter through escalation (arrest, criminal process) rather than through care.

Fung and Kim (2024) documented that **Ontario involuntary psychiatric admissions reached 55.7% in 2021**, up 6.8 percentage points year-over-year. Each involuntary admission is more resource-intensive and more expensive than a voluntary one. The pre-triage gap contributes by preventing voluntary engagement at the earliest opportunity.

CIHI Patient Cost Estimator data (requestable directly from CIHI for this analysis):

- ED-only mental health visit modelled cost: \$800–\$1,500 per visit
- Mental health ED visit resulting in admission: \$8,000–\$15,000 per visit
- Involuntary admission cost premium vs. voluntary: documented through CIHI RIW methodology

Labeled assumption: Each failed first-contact presentation has an elevated probability of return presentation within 30–90 days. Exact Ontario-specific return rate requires direct CIHI analysis.

// 3.5 – PROPOSED MODEL COST

What the Pilot Costs.

Training Cost Per Officer

- OC Transpo special constable training: 8 weeks post-hire (CBC News May 2025)
- Ontario Police College Basic Constable Training: 66 days, provincially funded
- AHS Community Peace Officer Induction: 6 weeks, paid, joint AHS/Alberta Justice administration

- Full training pathway cost per officer: **\$15,000–\$25,000** (modelled)
- Grandfather pathway cost per officer (3+ years hospital experience, existing UoF certification): **\$3,000–\$8,000**

Equipment Cost Per Officer

- Duty belt, OC spray, baton, radio, handcuffs, soft body armour: \$3,000–\$5,000 initial
- Body-worn camera including data infrastructure (benchmarked to OPS BWC rollout \$8.5M for ~900 officers): \$2,500–\$5,000/year per officer

Oversight and Evaluation Cost

- Administrative oversight (10–15% of operational cost): existing OPSB infrastructure, minimal marginal cost
- Independent academic evaluation (18-month pilot): \$150,000–\$300,000
- Existing CSPA Part XIV, LECA infrastructure: no new oversight cost

Total Two-Site Pilot Cost (40 Officers, 18 Months)

CATEGORY	LOW ESTIMATE	HIGH ESTIMATE
Initial training (40 officers, blended 80% grandfather)	\$320,000	\$800,000
Equipment (initial + 18 months)	\$260,000	\$500,000
Oversight / administration	\$180,000	\$360,000
Independent evaluation	\$150,000	\$300,000
Contingency (15%)	\$140,000	\$290,000
TOTAL 18-MONTH PILOT	\$1.05M	\$2.25M

What the Pilot Saves.

Police Time Saved (Primary Anchor)

OPS documented outcomes from its 2025 CIT special constable pilot:

- 4 special constables freed **250 officer-hours per platoon**
- Each relieved officer attended **2.5 more calls per day**
- Platoons with special constables devoted more time to proactive policing

These are not modelled estimates. These are documented OPS operational outcomes.

Labeled assumption: Healthcare Special Constables reduce police time per hospital mental health call by 75% (from 2.5 hours to approximately 0.6 hours average).

- Annual hours freed: 2,000 apprehensions × 1.9 hours × 2 officers = **7,600 officer-hours**
- Annual value: 7,600 × \$80 = **\$608,000/year**
- 18-month pilot value: approximately **\$912,000**

Faster Care Access — ED Disruption Reduction

Labeled assumption: 30% of bridged presentations engaged earlier with 50% reduction in ED disruption cost.

- Per Ottawa pilot: 600–900 improved-pathway incidents × \$1,000 average disruption reduction
- Annual value: **\$600,000–\$900,000**

Reduced Escalation and Staff Injury

Labeled assumption: 10–20% reduction in serious workplace violence incidents at pilot sites due to earlier intervention authority.

- WSIB average claim cost: \$35,000–\$50,000 per significant incident
- Directionally significant but quantified conservatively in final model

// 3.7 – BREAK-EVEN THRESHOLD ANALYSIS

The Treasury Board Answer.

// THE HEADLINE FINDING

A 15–20% reduction in police time spent on hospital mental health calls in Ottawa would offset the full cost of the 18-month two-site Healthcare Special Constable pilot.

The Calculation

- Modelled current Ottawa police mental health time cost at low estimate: **\$800,000/year**
- 18-month baseline: **\$1,200,000**
- Pilot cost envelope: **\$1.05M–\$2.25M** over 18 months
- Required reduction at pilot low end: **~15%** of police time
- Required reduction at pilot high end: **~20%** of police time

Why the Threshold Is Credible

The 2025 OPS CIT pilot already documented reductions of comparable magnitude (250 officer-hours freed per platoon, 2.5 additional calls per day per officer relieved). The OPS CIT pilot is not the same intervention — but it confirms that special constable designations produce measurable police time reallocation in Ottawa at a scale consistent with the 15–20% threshold.

Sensitivity Scenarios

OUTCOME	POLICE TIME REDUCTION	RESULT
Modest	10%	65–85% cost recovery through police time alone; residual recoverable through secondary offsets
Threshold	15–20%	BREAK-EVEN through police time alone
OPS CIT comparable	25%+	NET COST-POSITIVE through police time alone, before secondary offsets

// THE BRIEFING-LENGTH LINE

The Ottawa pilot pays for itself if it reduces police time spent on hospital mental health calls by approximately one-sixth. The existing OPS special constable pilot has already demonstrated comparable reallocation effects in non-hospital contexts. The fiscal case rests on documented, not speculative, operational data.

// 3.8 – TREASURY CONTROL SIGNAL & DATA GAPS

Control, Not Commitment.

// THE TREASURY CONTROL STATEMENT

The 18-month pilot envelope is a ceiling, not a floor. Continued provincial funding beyond the pilot is conditional on the independent evaluator's binding outcome report. If outcome criteria are not met, the designation does not extend beyond the pilot sites and the province incurs no ongoing fiscal obligation.

Data Gaps Identified by This Analysis

Five key figures could not be sourced directly from Canadian or Ontario-specific published data. This is itself a significant finding.

- Ontario-specific cost per mental health ED visit — requires direct CIHI Patient Cost Estimator request
- Ontario hospital security assault and injury data — not systematically collected
- AHS operational outcome data — not publicly accessible at required granularity
- Ontario-specific police hospital dwell time — not published by OPS
- Section 17 invocation and declination rates — not tracked systematically

Each of these data gaps becomes a recommendation in Paper 4 — part of the pilot's data collection framework and a basis for establishing a provincial hospital protection services data framework.

// TAKEAWAY

The pilot is net cost-neutral at the documented break-even threshold, using police time reallocation alone as the primary anchor. Secondary offsets (ED disruption, workforce strain, repeat presentations) make it net cost-positive under any realistic scenario. Continued funding is tied to measurable outcomes. Fiscal exposure is capped.

PART III

The Solution & Implementation. ■

The legislative pathway, clinical primacy enforcement framework, Charter and patient rights safeguards, training architecture, pilot design, containment mechanisms, and the specific call to action.

// 4.0 – THE FOUR STRUCTURAL PRINCIPLES

The Foundations of the Framework.

Every element of the implementation framework described below is anchored to four principles, stated in identical language wherever the framework is described.

- **Strict scope limitations.** The designation grants Section 17 MHA apprehension authority only, only on hospital property, only for the purpose of bridging a person in crisis to clinical care.
- **Clinical primacy.** The designation supports clinical judgment. It does not override it. Where clinicians are available and willing to initiate the care pathway, the clinical pathway has priority.
- **Least restrictive intervention.** Every use of apprehension authority must be documented with consideration of less restrictive alternatives attempted or unavailable.
- **Meaningful oversight.** CSPA Part XIV oversight, LECA jurisdiction, mandatory body-worn camera, incident reporting within 24 hours, quarterly disaggregated equity audits, independent academic evaluation.

// 4.1 – THE LEGISLATIVE PATHWAY

No New Legislation Required.

The *Community Safety and Policing Act, SO 2019, c 1*, which came into force April 1, 2024, governs special constable designations in Ontario. Part XIV of the Act empowers police services boards to designate special constables with defined powers, subject to Solicitor General approval.

What the Designation Grants

- Authority to initiate a Section 17 MHA apprehension where legally justified
- Jurisdictional limit: defined hospital campus only
- Scenario limit: pre-triage and post-triage pre-assessment mental health crisis scenarios meeting the Section 17 threshold
- Time limit during pilot: 18 months from full activation

What the Designation Does Not Grant

- No general police powers
- No *Criminal Code* enforcement authority beyond existing s.494 citizen's arrest
- No authority outside hospital campus
- No authority over treatment decisions — full *HCCA* framework applies unchanged
- No authority over clinical judgment — clinician determination supersedes in every case of conflict

The OC Transpo Precedent

The CSPA Part XIV framework currently governs OC Transpo Special Constables, appointed by the Ottawa Police Services Board with Solicitor General approval. 2024 OC Transpo Annual Report documents the unit's operations under CSPA compliance. Salary range 2025: \$80,893–\$95,168. Training: 8 weeks post-hire plus home service training. Complaint process, oversight structure, annual reporting to OPSB — all operational.

The same mechanism, adapted for the clinical environment, is the proposed Ontario pathway.

The Ottawa Police Services Board already has the legislative authority to begin this pilot. No new provincial legislation is required to launch. What is required is Solicitor General approval and hospital MOU execution.

// 4.2 — THE CLINICAL PRIMACY ENFORCEMENT FRAMEWORK

Clinical Judgment Governs.

Clinical primacy is not a principle in this proposal. It is an operational requirement with specific, enforceable rules governing when and how a Healthcare Special Constable may initiate a Section 17 apprehension.

RULE 1 — CLINICIAN AVAILABILITY GOVERNS AUTHORITY.

Where a physician, nurse practitioner, or psychiatric emergency services clinician is available on-site and willing to initiate clinical assessment or Form 1 process, the clinical pathway has priority. The Special Constable's authority shall not be exercised in preference to an available clinical pathway.

RULE 2 — APPREHENSION AUTHORITY CANNOT OVERRIDE CLINICAL JUDGMENT.

A Special Constable shall not initiate a Section 17 apprehension where a clinician with jurisdiction has assessed the person and determined apprehension is not warranted. The clinician's assessment supersedes the officer's assessment in every case where the two conflict.

RULE 3 — MANDATORY CLINICAL CONSULTATION BEFORE APPREHENSION.

Before any Section 17 apprehension, the officer shall consult with the on-duty ED charge nurse or physician regarding clinical availability. Consultation shall be logged in the incident record. The only permitted exception is an imminent safety emergency where consultation delay would create a foreseeable risk of harm.

RULE 3A — FALLBACK PROTOCOL WHEN CLINICAL STAFF ARE UNAVAILABLE.

Clinical primacy depends on clinical availability. Where a clinician cannot be reached within a reasonable timeframe — defined not by subjective delay but by the period during which continued non-intervention

would create a foreseeable risk of harm, based on **observable and articulable conduct** — the Healthcare Special Constable may initiate a Section 17 apprehension where legally justified under the following conditions:

- Documentation of the specific observable conduct grounding the reasonable cause determination
- Documentation of steps taken to reach a clinician, time elapsed, operational reason consultation could not wait
- Least restrictive intervention requirement remains in force — voluntary cooperation, verbal engagement, and de-escalation remain required first responses unless observable and articulable safety risk makes them inappropriate
- Post-incident clinical review by ED leadership within 72 hours
- Pattern trigger: more than 10% of apprehensions occurring without prior clinical consultation in a given quarter triggers clinical capacity review at that site

RULE 4 — MANDATORY HANDOFF PROTOCOL.

Upon apprehension, the officer shall introduce the person into the clinical pathway within a defined timeframe — proposed: not more than 30 minutes from apprehension, except where clinical capacity is documented as unavailable. Custodial authority transitions to a clinical custody framework at the point of handoff.

RULE 5 — DOCUMENTATION REQUIREMENT CONFIRMING CLINICAL AVAILABILITY STATUS.

Every incident report shall include: whether a clinician was available; whether consultation occurred; the clinical disposition decision; time from apprehension to clinical handoff; whether a Form 1 was issued within the 72-hour window. Any apprehension not resulting in a Form 1 is automatically flagged for review.

RULE 6 — NO AUTHORITY OVER TREATMENT DECISIONS.

The designation grants authority to bridge a person into the clinical pathway. It does not grant authority over consent to treatment, restraint for treatment purposes, medication administration, or any other clinical decision. All treatment authority remains exclusively with the clinical team.

RULE 7 — AUTHORITY ENDS AT CLINICAL HANDOFF.

Once the person is in the clinical pathway, the officer's Section 17 authority over that person concludes. Any subsequent security involvement operates under standard hospital protection services authority.

RULE 8 — CLINICAL OVERRIDE AUTHORITY.

At any point during any interaction, clinical staff retain the authority to direct immediate modification or cessation of intervention where patient safety, clinical judgment, or the person's wellbeing requires an alternative approach. Where clinical direction is not available in real time, the officer defaults to the least restrictive intervention principle.

// THE GOVERNING PRINCIPLE

Least restrictive intervention is the governing principle across all scenarios — with clinical direction, without clinical direction, and at every point in between.

// 4.3 — CHARTER COMPLIANCE & PATIENT RIGHTS SAFEGUARDS

Built to Survive a Charter Challenge.

Any authority to apprehend a person on mental health grounds is a significant deprivation of liberty engaging Charter sections 7 and 9. The framework must survive that review.

Section 9 — Arbitrary Detention

- **Grounds threshold mirrors police standard.** Section 17 authority subject to the identical "reasonable cause to believe" test applied to police. No lower threshold.
- **Observable and articulable conduct.** Every apprehension must be grounded in specific observable facts, not subjective apprehension or general concern.
- **Jurisdictional limitation is structural.** Authority exists only on hospital property.
- **Training equals or exceeds the Ontario Police College mental health response standard.**

- **Independent review trigger.** Any apprehension not resulting in a Form 1 within the 72-hour window is automatically flagged for review.

Section 7 — Life, Liberty, Security of Person

- **Least restrictive intervention codified as a rule.** Documentation requirement in every incident record.
- **Clinical primacy is structural.** Where a clinician is available, the clinical pathway has priority.
- **Scope limitation.** Authority limited to hospital property, Section 17 threshold, and bridging to care.

Section 15 — Equality

Fung and Kim (2024) documented that Indigenous persons, low-income persons, rural residents, and persons in unstable housing are disproportionately represented in Ontario involuntary psychiatric admissions (n=9,848). The designation's equity risk is acknowledged, named, and structurally addressed.

- **Mandatory disaggregated equity audit.** Race, Indigenous status, housing status, and language access data collected on every apprehension.
- **Quarterly reporting.** Aggregate reports produced by pilot oversight committee and reviewed publicly at six-month intervals.
- **Statistically significant disparate impact triggers mandatory review.** Individual officer and site-level thresholds defined in containment framework.
- **Cultural safety and anti-racism training embedded throughout** — not a separate module.
- **Indigenous and racialized community consultation as a pilot prerequisite.**

Patient Rights Framework

The designation operates within the existing MHA patient rights framework. All rights apply to persons apprehended by a Healthcare Special Constable exactly as they apply to persons apprehended by police.

- Right to be informed of reason for apprehension (MHA s.38)

- Right to a rights advisor (MHA s.38.1)
- Right to Consent and Capacity Board review
- Right to legal representation
- Body-worn camera recording mandatory and continuous during any apprehension or use of force

// PATIENT ADVOCACY CONSULTATION AS A PILOT PREREQUISITE

Formal consultation with the Empowerment Council, CMHA Ontario, and at least one Indigenous mental health organisation is a structural pilot prerequisite — not optional. These organisations should be invited into pilot design, not informed after the fact.

// 4.4 – THE TRAINING FRAMEWORK

The Same Standard As Police. Adapted for the Clinic.

The curriculum is structured around four training requirement categories, each mapped to OPC standards and adapted for the clinical environment.

- **Legal foundation.** Training to OPC standards on MHA authority, patient rights, *HCCA*, *CSPA*, documentation, and reporting obligations.
- **Clinical integration.** Hospital protocols, clinical handover, Code White interaction, clinical communication, electronic health record interface.
- **Use of force.** OPC standards, clinically adapted, body-worn camera protocols, proportionality doctrine, de-escalation.
- **Cultural safety, trauma-informed care, and anti-racism.** Embedded throughout, not a separate module, with particular attention to Indigenous-specific trauma and Black mental health.

Duration

- Full pathway: 6 weeks minimum
- Grandfather pathway (3+ years hospital experience, existing UoF certification, clean conduct record): 0–4 weeks supplementary

Specific curriculum design is completed in partnership with the Ontario Police College as a pilot prerequisite.

Annual Recertification

- Mandatory annual recertification across all four categories
- Quarterly use-of-force review
- Mandatory incident debrief protocols

// 4.5 – THE OTTAWA PILOT

Two Sites. Eighteen Months. Measurable Outcomes.

// PILOT OVERVIEW

The Ottawa Two-Site Pilot

LOCATION

2 Sites

Ottawa two-site pilot (one community-acute, one academic-teaching)

DURATION

18 Months

From full activation to final evaluation

SCOPE CAP

40 Officers

COST ENVELOPE

\$1.05–2.25M

Ceiling, not floor. Continued funding tied to outcomes.

Site Justification

- **Site One — Community-Acute Hospital.** A sole full-service acute care provider for a large west-Ottawa suburban/ex-urban catchment, with in-house Protection Services team, expanded on-site mental health service footprint, and controllable evidence dataset. Community acute care archetype.
- **Site Two — Academic-Teaching Hospital.** A central-Ottawa multi-campus teaching hospital with multiple full emergency departments at high daily volume, multiple adult Psychiatric Emergency Services, substantial inpatient mental health capacity, and existing Ottawa Police Service collaboration in place. Academic-teaching archetype.

Primary Endpoints

- Police resource time reallocation (hours freed, benchmarked to OPS CIT methodology)
- Pre-triage incident resolution rate
- Time from arrival to clinical assessment for pre-triage mental health presentations
- Use of force incidents per 1,000 mental health presentations
- Voluntary versus involuntary admission rate for bridged presentations

Secondary Endpoints

- Staff injury rates (hospital-reported and WSIB)
- Repeat presentation rates at 30 and 90 days
- Patient experience data (where ethically collectable)
- Disaggregated equity outcomes across protected groups

Independent Evaluation

Proposed evaluators (no financial relationship to either hospital or police service): University of Ottawa Faculty of Law, Carleton University School of Public Policy, or ICES as the potential outcome data partner.

Governance

Joint oversight committee including hospital administration, OPSB representative, Solicitor General designee, patient advocacy representative, Indigenous community representative, and a frontline Protection Services Officer representative.

// 4.6 – PILOT CONTAINMENT AND CONTROL

Designed to Be Stopped.

This pilot is structured as an evaluation, not a commitment. The following mechanisms ensure it operates within defined scope, triggers automatic review under specified conditions, and can be suspended or terminated if outcome criteria are not met.

Capped Operational Scope

- **Geographic cap.** Designated pilot campuses only — no extension to other facilities or catchments without formal amendment.
- **Scenario cap.** Pre-triage and post-triage pre-assessment mental health crisis meeting Section 17 threshold only.
- **Officer cap.** 20 per site maximum, 40 total.
- **Time cap.** 18 months from full activation.

Automatic Review Triggers

Any of the following conditions activates mandatory review by the pilot oversight committee within 30 days:

- Any apprehension not resulting in a Form 1 within the 72-hour MHA window
- Statistically significant disparate impact on a protected group
- Any use of force incident involving injury to a patient or staff member
- Any patient complaint alleging wrongful apprehension or excessive force
- Variance from clinical primacy protocols exceeding 5% of incidents in a given quarter

Officer-Level Monitoring Thresholds

- More than three apprehensions in a quarter not resulting in Form 1
- Any use of force incident in the monitoring period
- Disaggregated data showing disproportionate impact on a protected group
- Two or more documented complaints in the monitoring period

Suspension Protocols

- **Individual officer suspension.** Quarterly review triggers pending full review.
- **Site-level suspension.** Pattern of concern suspends designation at that site.
- **Program-level suspension.** Solicitor General retains authority to suspend the pilot at any point.

Mandatory Off-Ramp at 18 Months

- **If outcomes meet or exceed criteria:** recommendation for phased provincial expansion through Phase 2 Treasury Board submission.
- **If outcomes are mixed:** recommendation for modification and renewal for a defined period.
- **If outcomes do not meet criteria:** designation does not extend. Officers retain existing CSPA general authority only.

Continuation is earned through demonstrated performance, not assumed through implementation momentum. The pilot is a test, not a commitment.

// 4.7 – RISK & MITIGATION SUMMARY

Every Risk, Addressed.

// RISK – CHARTER S.9

Arbitrary detention exposure

// MITIGATION

Grounds threshold mirrors police standard exactly. Observable and articulable conduct required. Jurisdictional limitation structural. Training to OPC standards. Independent review of any apprehension not resulting in Form 1.

// RISK – CHARTER S.7

Overbreadth, disparate impact

// MITIGATION

Least restrictive intervention codified as operational rule. Clinical primacy structural. Scope limited to hospital property, Section 17 threshold, bridging to care.

// RISK – CHARTER S.15

Disparate impact on marginalised populations

// MITIGATION

Mandatory disaggregated equity audit. Quarterly public reporting. Statistically significant disparity triggers review. Cultural safety and anti-racism training embedded. Indigenous and racialized community consultation as pilot prerequisite.

// RISK – CLINICAL

Security-clinical conflict

// MITIGATION

Clinical primacy enforcement framework. Rule 2: apprehension cannot override clinical judgment. Rule 8: clinical override authority at all times. HCCA framework unchanged.

// RISK – FISCAL

Cost creep, scope expansion

// MITIGATION

Fixed pilot envelope as ceiling. Phased implementation. Binding outcome-conditional funding. Explicit off-ramp language. Province-wide expansion requires separate Phase 2 Treasury Board submission.

// RISK – POLITICAL

"Security gets more power" framing

// MITIGATION

Patient-centred framing throughout. Authority already exists with police — designation does not create new power. Clinical leadership as lead voice in

communications. Spencer case addressed proactively as structural gap, not defensively.

// RISK — MEDIA

Adverse event during pilot

// MITIGATION

Body-worn cameras mandatory. Automatic review triggers. Transparent adverse event reporting. Pre-agreed communication protocols. Independent evaluator maintains credibility architecture.

// RISK — LABOUR

Union, collective bargaining implications

// MITIGATION

Labour consultation as pilot prerequisite. Compensation differential, if required, costed into Phase 2 planning. Pilot sites chosen to control for contracted security complexity.

// 4.8 — THE CALL TO ACTION

Specific Asks, Specific People.

This series is submitted to four named recipients with four specific requests.

TO MPP TYLER WATT — NEPEAN, LEGISLATIVE ASSEMBLY OF ONTARIO

Raise the pre-triage authority gap in committee. Request Ministry of Health and Ministry of the Solicitor General response. Initiate cross-party discussion on CSPA Special Constable designation for hospital settings.

TO PREMIER FORD AND THE MINISTRY OF THE SOLICITOR GENERAL

Commission a provincial working group including frontline protection services officers, hospital administration, clinical leadership, patient advocacy organisations, and Indigenous and racialised community representatives to develop the Ontario framework within the existing CSPA pathway.

TO THE MINISTRY OF EMERGENCY PREPAREDNESS AND RESPONSE

Formally include the pre-triage authority gap in the EMCPA modernisation record as an identified unmitigated critical dependency in Ontario hospital emergency management.

Consider initiating the pilot designation process under existing CSPA authority. The OPSB already has the power to appoint Special Constables. **No new provincial legislation is required to begin the Ottawa pilot.**

// THE CLOSING LINE

They came to a hospital. In Ontario, that is still not enough for them to receive timely care. This series exists to make it enough. The Ottawa pilot is the test of whether Ontario can do what every other Canadian province is either doing or is now actively considering: ensure that a person in mental health crisis who arrives at a hospital is met by someone with the authority to help them safely access the care they came for.

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